

Case Study

Archives, Rhetorical Absence, and Critical Imagination: Examining Black Women's Mental Health Narratives at Virginia's Central State Hospital

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Abstract—Introduction: This article examines the rhetorical implications of archiving technical documents by studying the erasure of Black women's mental health narratives in Virginia's Central State Mental Hospital in the late 1800s and early 1900s. This article seeks to examine how historical mental health documents characterize (or fail to characterize) Black women and their mental health. **About the case:** We examine Black women's mental health experiences through absences in the annual reports from Central State Hospital in Virginia (formerly Central State Lunatic Asylum for Colored Insane). **Situating the case:** There is a dearth of work related to the unique experiences that Black women face when dealing with mental health challenges coupled with or compounded by a legacy of misogynoir.

Methods/approach: We offer an inventive approach for reading rhetorical absences and provide guiding questions for employing the critical archival inquiry methodology. **Results/discussion:** In taking on this endeavor to learn more about how Black women's mental health was represented in historical archives, we learned a great deal, not from the text on the page of the documents but from the text that was missing from those documents. **Conclusions:** Technical communication scholars, especially those with an interest in inclusion and justice, must adjust their methodological orientation and their approaches to historical and archival research to include an exploration of what is missing from the archives. Technical and professional communicators have a unique skill set that is ideal for reading through absences and erasures in both contemporary and historical documents.

Index Terms—Black women's mental health, Central State Hospital (CSH), Central State Lunatic Asylum for Colored Insane, critical archival inquiry method, mental health, mental health hospital.

Everybody knew what she was called, but nobody anywhere knew her name. Disremembered and unaccounted for, she cannot be lost because no one is looking for her, and even if they were, how can they call her if they don't know her name? [1, p. 274]

We begin this article with a transparent positionality statement that we understand as fundamental to the research we present. This is a purposeful rhetorical move that we deploy as a means of making ourselves and our experiences visible, memorable, and apparent. We are Black women technical communication scholars committed to the issues of inclusion and justice. Further, because we are Black women researchers, we understand the need for caution and care when

making public our situatedness and positionality (care and caution for ourselves and for others, past and present). This caution and care extends to the work that we undertook in conducting historical research about Black women's mental health in the US and the challenges of conducting this research with respect to Black women who lived during times when

1. Mental health care was still in development.
2. Black women's physical and mental health was not prioritized (or even really considered) by medical health professionals.

We mentally and emotionally steeled ourselves for what we might find in archival records dating back to the mid to late 1800s. We were not prepared, though, for the challenges of even gaining access to historical Black women's mental health records, which were publicized as digitized and available for review by scholars and researchers. We knew that we might find horror and hate in the historical archives (as they were related to Black women), but what does it mean when we (Black women) are largely missing altogether?

Initially, the purpose of this article was to present the findings of an archival examination and analysis of historical Black women's mental health

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Practitioner Takeaway

- Archival and historical records about Black women's mental health reveal a persistent history of silence and silencing about Black women's mental health, which continues today.
 - This history asks us to interrogate how we can reveal and center Black women in our archival work, understanding that absences are rhetorical, sometimes purposeful, and thus need to be addressed directly.
 - Historical records and data are always rhetorical, not neutral or apolitical, just as we have also come to understand that technical communication is not neutral or apolitical.
 - Recognition of Black women's narratives, histories, and lived experiences also requires valuing their epistemological traditions and contributions for the field of technical communication.
 - The critical archival inquiry approach that we present may have practical implication for implementing diversity, equity, and inclusion initiatives in the workplace and community.
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records, including records from the Central State Hospital (CSH) in Virginia in the late 1800s. Though records are available in the Library of Virginia's archival collection from 1874 to 1961 and are available in various formats (financial records, commitment papers, furloughs and bonds, and sterilization records), due to COVID-19 travel restrictions, we intended to limit the study to available digitized case books, which include information regarding treatments and progress reports. Unfortunately, after communicating with the researchers, who spent 10 years digitizing over 800,000 records from CSH (and saving these records from being destroyed), we discovered that recent policies had restricted our access to files less than 125 years old—all the records that we intended to examine.

In technical and scientific communication, it is not unusual for the researchers to struggle to gain access to archived records on issues that we find important. For example, Banks [2] described how the attention to quilts created by enslaved Blacks in the US, some of which are believed to have included coded routes and symbols to freedom,

has been limited to aesthetic analysis rather than examination of the purposes the quilts actually served and the cultural contexts from which they emerged. [2, p. 123]

In addition, though Williams [3] was able to examine Bureau of Refugees, Freedmen, and Abandoned Lands reports that documented federal agents' attempts to void illegal labor contracts between White landowners and Black children in the South, the agency's archives did not contain documentation of the contracts or the Black children who were bound by these illegal documents. Finally, Jones and Williams [4]

discussed obstacles encountered when attempting to validate and identify archived literacy tests and voter application forms used to disenfranchise Blacks in the US as well as civil rights' organization's manuals used to help Black voters resist disenfranchisement.

Although the archives of Black artifacts used to resist White supremacy are hard to find, it is often equally and purposefully difficult to find artifacts used by White institutions in the service of oppressing Black people. In the US, White institutions have had little interest in the ethical archiving of unethical practices. But despite the challenges, Galliet encourages archival researchers to share

back stories about the processes of discovering materials, the rationale and reasons for their selected narrative structure, and tales of rigorous research paired with the sometimes serendipitous nature of archival investigation that are critical to producing ethical and transparent scholarship. [5, p. 51]

Only after learning that we would be unable to access the records that we initially hoped to examine were we directed to the technical communication documents that we examine in this article: Central State Lunatic Asylum for Colored Insane mental health hospital reports.

As technical communicators, we are keenly and uniquely attuned to how power and people are often obfuscated under the guise of neutral, technical, and objective language. Further, we argue that technical communicators' expertise in both rhetoric and design positions them as skilled potential advocates for patients, users, and others [6], [7]. In the context of archival research, a focus

on historical technical documents from a technical communication perspective can reveal interesting clues about marginalized groups and multiply marginalized groups' lived experiences.

BLACK WOMEN AND THE RHETORIC OF THE ARCHIVE IN TECHNICAL COMMUNICATION

Like technical communication as a discipline, we assert that archives are not neutral, apolitical, or objective—even though they are often presented as such. Our interest in archival and historical records about Black women's mental health stemmed from our experiences and observations, as Black women scholars, that reveal a persistent history of silence and silencing about Black women's mental health. As we drafted this article, Oprah Winfrey conducted an interview with Meghan Markle, the Duchess of Sussex, in which Markle revealed that her mental health concerns had been silenced and went unaddressed by those around her [8]. Similarly, Michelle Obama recently described her struggles with depression. In fact, the former First Lady acknowledged that, among other personal, political, and societal pressures, racism and witnessing racist violence that played out in very public ways impacted her mental health. She stated,

We had the continued killing of Black men at the hands of police. Just seeing the video of George Floyd, experiencing that eight minutes. That's a lot to take on, not to mention being in the middle of a quarantine. [9]

Most recently, we witnessed racist backlash from media pundits and social media influencers when Simone Biles chose to protect her mental (and physical) health by withdrawing from some Olympic events. There is an undeniable connection between race, gender, and mental health. Moreover, this acknowledgement of the role of racism in terms of Black women's mental health is a contemporary recognition of the role that intersecting oppressions have and have always had on how Black women struggle with and cope with trauma that is both gendered and racialized (not to mention issues of class, access, and disability) [10].

Technical communication has long been interested in understanding, collecting, and analyzing technical documents as a way of historicizing the impact of the field in various contexts, spaces, and places [3], [4], [6], [11], [12]. In fact, technical communication scholars often look for documents developed outside the discipline of technical

communication. These documents include texts developed by individuals who would not have considered or do not consider themselves to be technical communicators. However, technical communicators understand the implications of the influence that procedures, practices, and perspectives that are mediated by seemingly mundane and unimportant documents can have in reinscribing, reifying, and re-envisioning the power of economic, political, legal, and social systems and institutions.

With the acknowledgement of the social justice turn in technical communication [12]–[17], scholars now, more than ever, argue that societal values about justice, equality, and inclusion are always embedded in these documents, processes, procedures, and sites of practices that are germane to the study of technical communication. This recognition brings with it glaring inequities and disparities between whose knowledges are recognized in technical communication, whose histories are considered relevant to a field-wide understanding of the foundations of technical communication, and whose pedagogical approaches, methodological practices, and applied scholarly frameworks are relegated to the margins of technical communication. It is here at the margins that we locate much of the history, knowledge, and work of Black women.

Black women's "issues" have not often been centered in academic or professional contexts, and technical communication is no different. Likewise, there is a dearth of work related to the unique experiences that Black women face when dealing with mental health challenges coupled with or compounded by a legacy of misogynoir. This article seeks to address some of these erasures by using the lens of the archive and rhetorical research methods to examine how historical documents characterize (or fail to characterize) Black women and their mental health. Extending Berkenkotter's work on narratives, technical communication, and psychiatry [6], we locate Black women's mental health experiences through absences in the annual reports from CSH in Virginia (formerly Central State Lunatic Asylum for Colored Insane), which was "the world's first Black mental health hospital" [18].

We recognize that whether Black women's narratives are included in archival and historical records, like the records in the CSH archives, is ultimately a rhetorical choice, underpinned by the refusal to recognize Black women as fully human. Ray's [19] characterization of hypothetical

substitution intersects with conceptualizations of the archive as rhetorical (a practice in invention). Ray posits that “observing rhetorical roads not taken reveals the important dimensions of what was done” and what was not done, who was included, and who was omitted [19, p. 50]. Further, Ray asserts that

the technique of hypothetical substitution, or the emphasis on absence, is rarely marked as a technique, but it is frequently implemented to illuminating effect in rhetorical analysis. [19, p. 50]

Ray uses as an example of substitution, a 2015 book, *Picture Freedom*, by Jasmine Nichole Cobb [20]. Ray explains, Cobb observes that free Black women in the antebellum urban North, as they created friendship albums, almost never portrayed the Black female body. Instead, she argues, they represented Black women’s bodies through floral motifs, drawing on the cultural resonance of flowers. [19, p. 50]

Black women’s bodies were absent. Ray notes that Cobb’s rhetorical strategy of “analytic linking of absence and presence” creates a “nuanced way of seeing the past” [19, p. 50]. Essentially, the absences expand the ways that we read an archive, with an eye to critical, imaginative, and inventive engagement, pushing us to question who is represented (or not) and why. Further, this linking of absence and presence also brings to the fore connections among audience, subject, and author, asking how absence can be used by marginalized groups as a way of re-envisioning (or even protecting) versus how absence is used to disappear or delegitimize marginalized groups.

For researchers interested in the issues of inclusion and amplification, this recognition of the connection between absence and presence is critical in that it offers a way of interrogating epistemological traditions that are not dominant and are often scrutinized as less valuable and less valid. In this sense, we see Dotson’s [21] ideas of silencing at play in literal and metaphorical ways. Dotson argues that testimonial quieting, drawn from the work of Patricia Hill Collins [22], occurs when “an audience fails to identify a speaker as a knower” [21, p. 243]. In terms of the archive, we can extend Dotson’s conceptualization of testimonial quieting to interrogate how patient narratives by Black women, though common in White mental health records as noted by Berkenkotter [6], are absent or inaccessible.

Though Collins and Dotson both discuss how “controlling images” of Black women in linguistic and dialogic contexts can point to what Dotson terms “epistemic violence,” Dotson is careful to note that a lack of knowledge is not at the core of this separation of Black women from knowledge and knowledge making. Rather, it is “the active practice of unknowing” that is also of consequence [21, p. 243]. In examining absences in the archive then, we see the disconnect between knowers and their knowledges and seek to make visible the ways that their knowledges are relevant and have been ever-present (historically and contemporarily), even if they have not been recognized. Especially as Black women scholars, we deeply appreciate and embrace this reconnection and recognition of being seen even in our disappearances.

Our interrogations into the archive and into the historical and contemporary representation of Black women’s mental health are productive, resistant, and subversive but not wholly unfamiliar. Black folks (and particularly Black women and children) have a long and tumultuous history of residing in absences and silences. Historically, Black women and children have been disappeared from archives, often being recognized only in their connections to men, capital as slaves, or labor [23]. These histories of systemic erasure have often required that Black women scholars question and interrogate. If we fail to “locate” ourselves in historical narratives, who will do this work on our behalf?

As Walters notes,

Black writers from multiple geographic locations have long queried dominant archives, revising the ways Black people have been represented in the legal and hegemonic discourses of the west. [24, p. 3]

Further, Walters asserts that

cultural workers such as Harriet Jacobs, W. E. B. Dubois, Arthur Schomburg, Robert Hayden, and Sherley Anne Williams, to name just a few, have long taught us to be skeptical of dominant archives. [24, p. 3]

This skepticism cultivated in cultural, political, economic, legal, and embodied erasure requires the type of interrogating and reading between the lines that Ray [19] champions. What is more, however, is not only the skeptical (or critical) interrogations fundamentally but the critical imagination and the inventiveness necessary to move beyond seeing only absences to the revelation and even the creation of the narratives of the lived experiences of

those disappeared. Reading an archive for absence and silence does the hard work of locating and critically imagining the experiences and narratives of those missing from the historical record.

We use Royster's concept of critical imagination [25] paired with Ray's productive and inventive reading of absences as "rhetorical roads not taken" [19, p. 50]. One important aspect of Royster's critical imagination is

to think between, above, around, and beyond [this] evidence to speculate methodically about probabilities, that is, what might likely be true based on what we have in hand. [25, p. 71]

Knowing what we know about Black womanhood and the ways that Black women have long been disappeared, dehumanized, and disremembered in this country, we can only imagine what would have been found in the missing narratives of Black women institutionalized at CSH from the 1840s through the 1940s.

To that end, we wanted to ask the following questions.

- What were the stories of Black women at CSH and what were their experiences?
- Why were they there, and how were they treated?
- Most important, how did they survive, physically and psychologically?
- What narratives and stories did these Black women have to tell themselves to aid in their own survival?
- Finally, what stories were told about these women and why were these stories omitted from the historical records at CSH?

METHODS: IDENTIFYING OMISSIONS IN THE ARCHIVES

Narrative is key to our methodology. In the following, we share our own experience in undertaking this work, acknowledging how we encountered and addressed absences (Ray's linking absence with presence) and how we used critical imagination (Royster and Kirsch) to read the archival absences anew.

In the fall of 2020, we contacted Dr. King Davis, Mike Hogg Professor Emeritus with the University of Texas at Austin's Institute for Urban Policy Research, to request access to records from the CSH in Virginia. Dr. Davis, who currently works as a Research Professor with the School of Information at the university, informed us that access to over

800,000 digitized files, which he digitized during a 10-year funded research project, was now restricted. He stated that although the project's initial goal was to make the documents available to the public, the State Library of Virginia owns the records, and to adhere to the Health Insurance Portability and Accountability Act (HIPAA), the State Library had "recently changed their policy to restrict access to records from 75 to 125 years old or more, which means the records" are no longer available to scholars [37]. We were disappointed and at a loss as to how we might find out more about one of the few historical archives for Black women's mental health and institutionalization.

Although Dr. Davis's reply to our request confirmed that not only would we not have access to the physical files due to travel restrictions resulting from the COVID-19 pandemic but that we would not even have access to the digitized files due to HIPAA restrictions. After recommending that we access the available online annual reports instead of the hospital records, Dr. Davis also made the following note regarding the danger of the complete erasure of this important archive of CSH's physical mental health records:

We did not digitize the actual psychiatric or medical treatment records for any of the patients admitted. Those records remain on microfilm in the hospital and are at substantial risk because of the brittleness of the microfilm and the extreme temperatures in the medical records room where they are kept. Summer temperatures exceed 100 degrees many days. We are searching for additional funding to support copying these records since they are the first of Black folks admitted to a psychiatric facility soon after Emancipation [37].

As we came to realize that due to COVID-19, HIPAA, other legal concerns, and the risk of damaging already deteriorating microfilm, we would be unable to conduct the intended archival research by reviewing actual historical medical documents. We were also forced to acknowledge that we were experiencing common and often encountered limitations on accessing limited historical information about Black folks in the US. Historical records and archives about Black folks in the US are notoriously limited and incomplete.

In *Archives of the Black Atlantic: Reading Between Literature and History*, Walters acknowledges the need to read "between the data of the archive" due to the "meager traces recorded in the archives of

the slave trader, colonizer, or court room” [24, p. 1]. This meagerness, this disappearance of, or, more appropriately, the **disappearing** of Black lives and lived experiences has been persistent and consistent, making hard work for researchers, scholars, or individuals interested in tracing the Black experience in the US across the centuries. However, Walters is hopeful, asserting that creative writers in particular

teach us to read the archives of history anew, probing between the documents for stories left untold, questions left unanswered and freedoms enacted against all odds. [24, p. 7]

We too are hopeful and seek to engage with Walters’ optimist view of reading archives for stories untold.

So given Davis’s reply, we gathered ourselves and decided that instead of completing the archival research as we had hoped, we would shift our focus to a more critical examination of the “disappearing” of Black women and Black women’s mental health concerns from the archive and from academic scholarship. Even though this was not the approach that we had intended to take for this study, we see our discoveries as productive, helping us to include creative narratives about our experiences to draw attention to a larger concern for Black women’s theoretical and methodological work in technical and professional communication (TPC). In addition, this work, we hope, will seed the work of an archival project once we are able to physically visit and access the CSH archives after the pandemic and before even these files are completely unavailable because of inappropriate preservation or intentional destruction.

Further, we took Davis up on his recommendation that we try instead to find CSH’s annual reports online, which he believed to be the **only** documentation accessible to researchers. When we searched, however, we hit yet another wall. Frustratingly, we did not find the annual reports on the websites that Davis had recommended, but we **did** find annual reports from 1891 to 1936 through the HathiTrust Digital Library, which the organization describes as “a not-for-profit collaborative of academic and research libraries preserving 17+ million digitized items” [26].

In our search for research conducted by scholars who have accessed CSH’s records, we found very few published studies. Of the few, Brooks analyzed the commitment records of a sample of 105 out of 922 patients admitted to the mental health hospital

between 1874 and 1881; though 922 patients were admitted during this period, only 429 commitment papers (for fewer than half of the patients admitted) were available at the time of the study [27, p. 74]. Of the sample, which included two gender categories of 48 women and 57 men [27, p. 74], Brooks found that

The reasons that sent people to Central Lunatic Asylum for the Colored Insane (CLA) fell into four categories: Mental Health Issues, Cultural Issues, Dangerous Behavior, and Work Related Issues While fifty-two individuals may have been mentally ill, due to suicidal tendencies, hallucinations, and other symptoms, as many as forty-six, (or forty-seven percent) may have been committed for cultural differences, violent self-defense, and resisting working for Whites. Some detainees believed that they had a right to own the land they had worked as slaves; others seized such property as they thought belonged to them. Still others preached and prayed through the night or feared that a conjurer’s spell had tricked them. All of these beliefs and practices sent individuals to the asylum. [27, p. 76]

Although we found Brook’s analysis of **why** patients, irrespective of gender, were admitted to the hospital useful, in this study, we are interested in identifying **how** hospital administrators and physicians communicated about the treatment and care of Black women in CSH’s annual reports. The “how” is often communicated in narratives and case histories, to which we did not have access but which would have been immensely helpful. Berkenkotter’s [6] work on psychiatric narratives and case histories makes clear the role that the narratives of medical personnel (in this specific case, professionals in psychiatry) play in patient treatment and medical records. As Berkenkotter notes, a “double narrative” (constituted of the patient’s own narrative coconstructed with the narrative of the psychiatry professional) is a

linguistic and semantic device the therapist uses to recontextualize the patient’s narrative of personal experience into a more encompassing narrative framework that has been highly codified within the psychiatric profession, including the mental health clinic. [6, p. 2]

Although this is interesting and helps to build our understanding of the genre of a psychiatric case history, Berkenkotter notes that these “patient tales” were primarily identifiable in “Anglo-American asylums in the late eighteenth and nineteenth centuries” [6, p. 2]. What we know

about the status of Black women during those times is telling in that we are left to wonder what “patient tales” formed the foundation of case histories of Black women in mental health. In fact, one can assume that because Black women were not seen as fully human, were systemically and systematically oppressed, and were not allowed to advocate for their own care, the patient tale is likely one that does not exist in relation to Black women’s mental health case histories.

In this way, data from CSH’s commitment papers, not disaggregated by gender, show an incomplete picture of how Black women may have experienced mental health care at the facility. Unfortunately, this is not a surprising discovery. Black women and their mental health records have effectively disappeared behind decontextualized numbers of patients (rather than **recontextualized** in case histories in the way that Berkenkotter describes). To draw out a richer understanding of the story behind the numbers, we moved on to examine the archival documents that we were able to access. And, in doing so, we were intent on resisting being “disremembered and unaccounted for” [1, p. 274] and insistent on locating voices like our own in the archives.

Of course, the methods for locating voices like our own in the archives are not new in technical communication research. When examining historical records and documents about Black people in the US, technical communication researchers have identified and analyzed historical records regarding Black entrepreneurship and labor [3], [11]; Black home buying [28]; and Black voting and disenfranchisement [4]. These documents were written using racially neutral language but were purposefully and intentionally oppressive. To identify what is missing from technical communication artifacts that address the economic, health, social, and political lives of multiply marginalized people, technical communication researchers should consider employing a critical archival inquiry approach by responding to the following questions.

- What are the intersectional vulnerabilities of the population discussed in the document (for instance, race, gender, and class)?
- In what ways does the organization/institution (via the author) identify or address vulnerable groups? Or are these groups part of the audience? Or are marginalized groups simply the “subject” of institutional documents?
- What does the historical record say about the relationship between the author and the

audience? Or what does the historical record say about the relationship between the author and the vulnerable population?

- In what ways does the content match the historical record?
- Of what we know of the historical record, what content is available? Describe the content.
- Of what we know of the historical record, what is omitted from the archive?
- What rationale might the author have for leaving out this information?

DESCRIPTIVE ANALYSIS OF CENTRAL STATE HOSPITAL’S ANNUAL REPORTS 1891–1936: WHAT CONTENT IS AVAILABLE?

As technical communicators are interested in historical documents, we understand that design, form, and function of documents often tell a story, including whom the document was created for, who is missing from the document, and how a document was used. This section provides some background regarding what the reports from the hospital looked like and what information they included; and again, whose voices, perspectives, and knowledges are privileged. These voices, perspectives, and knowledges are what remain for analysis and point to the ways that archives are not only subjective but also subject to both readers’ and historians’ rhetorical invention [29].

In general, in our initial review of the content of CSH’s annual reports, we found that the content and organization of the reports remained mostly the same throughout the years that we examined for this study. For example, the report for fiscal year ending in 1892 [30] includes a title page listing the fiscal year and name of the hospital; a page listing the board of directors and their terms of service, and the executive committee and officers; an index of tables page; and a page titled “Report of Board of Directors,” which identifies the Governor of Virginia as the audience for the report. This was not unexpected for the genre of an annual report, and we found the front matter to be largely unremarkable. However, it was clear to us that the author and audience of the report were likely White men in powerful positions (a state governor, a board of directors, and an executive committee). These details matter for who is made (in)visible in the context of the report, and we find it valuable to again consider the rhetorical role of author, audience, and subject.

The body of the 1892 report [30] begins with a four-page narrative, written by Randolph

Bardsdale, Physician and Superintendent. This narrative is Bardsdale’s alone and does not include comments or commentary from other sources. The first section of the 48-page report is titled “Report of the Superintendent to the Board of Directors” and includes information about the administration of the hospital, including sections devoted to amusement and employments, integrating anecdotes from Bardsdale’s point of view that note, for example, that “The dance on Thursday afternoons is enjoyed, especially by the female patients” [30, p. 8]. In addition, Bardsdale’s narrative provides more quantitative data such as the number of patients employed in the laundry, kitchen, bakery, dining rooms, wards, sewing-room, garden, and farm. We are unsure whether this was true “employment” or forced labor.

In addition to this quantitative data, the report includes the following sections:

- Information about the farm and garden, which summarizes the report of the hospital’s farmer
- Details on finances
- Proposal for a new hospital building, described as “a new addition to the female department”
- Rationale for the addition that states that

from all the information I can get there are at least one hundred and twenty five colored people within the State, who have been adjudged insane, and the authorities in the various counties and cities are constantly clamoring for room for their respective poor unfortunates. [30, p. 9]

- Details about repairs and improvements, and recommendations, which describe the need to paint part of the main building

What is most interesting here is that Bardsdale’s narrative represents a sole perspective of the mundane and operational occurrences in the hospital. Aside from summarizing the hospital farmer’s report, this report’s only insight is provided by Bardsdale. Bardsdale’s narrative makes comments on the feelings and emotions of the Black women in the hospital, but there are no ways to confirm that Bardsdale’s perception of how much, for instance, the female patients enjoyed a Thursday afternoon dance, is accurate or false. Beyond this short narrative, we found that the rest of the report consists solely of primarily quantitative data tables in the appendix, which are listed in Fig. 1. (Note that the tables contained in the figures in this article have been transcribed

INDEX TO TABLES	
I.	Movement of population.
II.	Admission and discharges from beginning.
III.	Number of each age when admitted during year.
IV.	Number of each age from beginning.
V.	Nativity of patients admitted.
VI.	Residence of patients admitted.
VII.	Occupation of those admitted.
VIII.	Civil condition of those admitted.
IX.	How admitted.
X.	How supported.
XI.	Supposed cause of insanity of all admitted.
XII.	Form of disease of those admitted,
XIII.	Complications of those admitted.
XIV.	Number of attacks of those admitted.
XV.	Duration of insanity before entrance of those admitted.
XVI.	Recovered of those attacked at the several ages.
XVII.	Duration of treatment of those recovered during the year.
XVIII.	Whole duration of disease of those who recovered in the year.
XIX.	Form of disease of those recovered from the beginning.
XX.	Causes of disease of those recovered from the beginning
XXI.	Nativity of those recovered from the beginning.
XXII.	Deaths and causes.
XXIII.	Ages at death.
XXIV.	Ratio of deaths for the year.
XXV.	Duration of disease of those who died within the year.
XXVI.	Remaining in asylum at end of year—ages.
XXVII.	Remaining at the end of the duration of disease.
XXVIII.	Remaining at end of the year—prospect.
XXIX.	Operation of the asylum for the past six years.
XXX.	Admission per centum for various causes for the past six years.
XXXI.	Employment of patients in the year.
XXXII.	Officers and employees.
XXXIII.	Financial history.

Fig. 1. Index to tables in Bardsdale’s 1892 *Annual Report* [30].

because the poor quality of the images we examined resulted in poor photographic reproduction.)

In later CSH annual reports, there are some changes in superintendents and other staff, but the content of the annual reports from fiscal year 1891 through 1899 remains mostly the same with small adjustments. By 1898, when William Francis Drewry, Physician and Superintendent, wrote the narrative of the report, Drewry [31] decreased the

STATISTICAL TABLES	
I.	Movement of population for the year.
II.	Number at each age when admitted during the year.
III.	Number at each age from the beginning.
IV.	Nativity of patients admitted.
V.	Residence of patients admitted.
VI.	Occupation of those admitted.
VII.	Civil condition of those admitted.
VIII.	How supported.
IX.	Alleged cause of insanity of all admitted.
X.	Form of insanity of those admitted.
XI.	Number of attacks of those admitted.
XII.	Alleged duration of insanity before entrance of those admitted.
XIII.	Several ages when insanity began in those who recovered.
XIV.	Duration of hospital treatment of those who recovered during year.
XV.	Form of insanity of those who recovered.
XVI.	Death and causes.
XVII.	Ages at death.
XVIII.	Duration of insanity of those who died within the year.
XIX.	Duration of insanity of those remaining at the end of the year.
XX.	Prognosis (prospect) of those remaining in hospital at end of year.
XXI.	Operation of the hospital from the beginning.
XXII.	Ratio of per cent. in each year of deaths, recoveries, &c.
XXIII.	Employment of patients during year.

Fig. 2. Index of tables in Drewry's 1898 *Annual Report* [31].

number of tables from previous reports and increased narratives to include the following sections:

1. Admissions
2. Discharges
3. Mortality
4. Statistical
5. Applications and admissions
6. Financial
7. Medical treatment
8. Pathological
9. Trained nursing
10. Employment and amusements
11. Religious services
12. Farm and garden

13. Improvements and repairs
14. Fire protection
15. Needed improvements
16. Conclusion

In Drewry's 1898 report, the index of tables section was changed to "Statistical Tables," better indicating the quantitative nature of the data in the tables; however, his 1898 report includes 10 fewer tables than the 1892 report. As Fig. 2 indicates, additional content was added to the report, including the inclusion of shorter "reports" at the very end of the document. Assumptive narrative sections, such as "Report of Farm" and "Report of Carpenter's Work," are included as lists of services performed but do not provide the actual narrative

of the reports. Drewry's superintendent's reports ranged from 58 to 126 pages in length, with the bulk of the content still made up of tables, rather than narratives.

Even though Drewry [31] relied heavily on quantitative data like Barksdale, his predecessor, he did, as noted above, include more narrative than Barksdale. Drewry's reports were more detailed in narrative than Barksdale's and included more racist commentary. For example, in a subsection of Drewry's 1903 narrative, "Increase of Insanity and Causes," the physician stated that increases in cases of "insane" Black people were attributed to "race degeneration." Drewry's discussion of the implications of race degeneration is reproduced in its entirety in Fig. 3.

Even as Drewry's racist ideas about "race degeneration" provide a basis for his annual narrative, reading this archival document from a critical and historical perspective, we can read racialized economic, social, and political motivations into the narrative. As postbellum Jim Crow takes hold in the US, we can track Drewry's noted increase on the Black "insane." In his narrative, Drewry wonders, "How can this increase in insanity in the negro during the past thirty-five years be accounted for?" [31, p. 11]. And, although Drewry provides his own racist explanation, we know that institutionalization of Black people has always been a means of physical, social, economic, and political control.

In fact, Wingerson [32] notes that racist, pseudomedical, and pseudoscientific eugenicists often touted freedom from enslavement as a root cause of insanity in Black populations. Specifically, Wingerson notes that the 1840 census

concluded that one in 144.5 African Americans from free states were insane or idiotic, compared to one in 864 whites. In slave states, however, one in 1,558 African Americans were insane or idiotic. [32, p. 15]

In fact, Wingerson also points out that after reviewing the 1840 census, Senator John C. Calhoun ecstatically declared, 'here is the proof of the **necessity** of slavery. The African is incapable of self care ... and sinks into lunacy under the burden of freedom' and further notes that slavery **protects** from mental illness and death [32, p. 15, emphasis added]. We see echoes of this deeply ingrained racism throughout Drewry's 1903 narrative report.

CRITICAL AND RHETORICAL ANALYSIS CENTRAL STATE HOSPITAL'S ANNUAL REPORTS: WHAT IS OMITTED FROM THE ARCHIVE?

The CSH annual reports from 1899 to 1936 contain approximately 2540 pages, including approximately 277 pages in reports for 1891–1899, 929 pages for 1900–1909, 691 pages for 1910–1918, 391 pages for 1919–1929, and 251 pages for 1930–1936. Throughout these five sets of annual reports, there are missing years. For example, years 1893, 1894, and 1897 are missing in the set for 1892–1899. In the reports, all patients are referred to as "negros" when speaking of the collective group. To identify discussions that focused exclusively on Black women patients, we searched the reports using keywords that were commonly used to describe women or women's mental health during this period: "woman," "women," "female," "females," "wife," "invert," "sexual inversion," "widower," "children," "girl," or "girls." Although no references to "woman" or "women" appeared in any of the tables from 1892 to 1899, there were references to "women" in the Superintendent's reports and references to "females" in the tables. Most of the tables that reference patient data are presented with columns dedicated to "males" and "females"; so there is a reference to females in almost every table related to admissions of patients.

However, most of the tables in the reports from 1892 to 1899 include numerical data or short textual descriptions (i.e., hometown, age, medical conditions, and causes of psychosis) about each newly admitted patient. Fig. 4 references the law that allows "persons suspected of being insane" to be committed. In other words, the report writers' discussion of how patients are admitted is vague and legalistic in comparison to the specificity provided regarding other descriptions of the patient population. There is no reference to who committed these patients and under what circumstances. Instead, the "How Admitted" table provides us information about the law that authorizes judges, physicians, and "other witnesses" to commit patients. By 1898, this table was no longer included. One can only imagine whether this law applied to Black women in 1898. The archive provides no definitive indication.

Finally, although most tables describe the number of women and men admitted or discharged or other demographic information (i.e., age, employment, and residence), other tables include details about the causes of mental illness of patients admitted by gender (see Figs. 5 and 6). One of the most

REPORT OF THE CENTRAL STATE HOSPITAL

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INCREASE OF INSANITY AND CAUSES.

A few observations on the increase and the causes of insanity in the negro may not be inappropriate. From the best available information insanity, and tuberculosis as well, was almost unknown among ante-bellum negroes. In 1860 only 25 insane negroes were in the State Asylum at Williamsburg. In February, 1870, when the State took control of the Howard's Grove Hospital, near Richmond, there were 123 insane negroes reported in the State. To-day there are reported 1,074. In 1860 the ratio of insane negroes to the general negro population was 1 to 7,000; in 1870, 1 to 3,000; in 1880, 1 to 1,950; in 1890, 1 to 940; in 1900 1 to 640. Between 1870 and 1880, 495 first admission cases were received into the Hospital, an average of 49 a year; during the next decade, the number was 1,100, a yearly average of 110; from 1890 to 1900, the number of first admissions reached 1,512, a yearly average of 125. From October 1st, 1900 to October 1st, 1903, 745, or an average of 248 a year, were received.

How can this increase of insanity in the negro during the past thirty-five years be accounted for? The increase, in my opinion, is a result as well as an indication of race degeneration. During slavery the negroes were usually under strict but kind discipline, and moral and regular habits were enforced; they lived in clean and hygienic surroundings, were well fed and nourished with wholesome food, were comfortably clothed, led systematic, regular lives, had no mental worries and anxieties, and no responsibilities, to speak of. In short, their habits, surroundings and every-day lives were all conducive to robust physical health and strength and mental contentment. With emancipation came unbridled appetites and passions, dissipation, excesses and vices, bad habits, violation of the laws of health, irregular living, indolence, privation, etc., and as a consequence, the negro soon began to lose vitality, his constitution became weaker, and hence he became a more ready prey to diseases such as insanity and consumption. Depraved and immoral habits have brought about more widespread venereal diseases to be transmitted to the third ~~and~~ fourth generation. The mental strain and responsibility incident to caring for himself have also been significant factors in the causation of insanity. Being of an emotional and impressionable temperament, the wear and tear of excesses and the pressure of excitement have been too much for the negro. Formerly heredity was not a factor in the causation of insanity in the race, but now that predisposing cause has become one of considerable consequence.

Fig. 3. Drewry's 1898 *Annual Report* discussion of the "Increase of Insanity and Causes" [31, p. 11].

The law requires all persons suspected of being insane to be examined before three justices, summoning a physicians and other witnesses, the justices requiring the commitment, &c. See Code of 1887, 1669, 1670, 1671, &c.

Fig. 4. Note on how patients were admitted from the 1896 *Annual Report*.

TABLE XI. SUPPOSED (EXCITING) CAUSE OF INSANITY OF ALL ADMITTED.			
CAUSES	Males.	Females.	Total.
Abortion	-----	5	5
Apoplexy	2	-----	2
Bright's disease	-----	1	1
Blow on head	53	13	66
Burn	1	-----	1
Business anxiety	7	-----	7
Brain fever	4	2	6
Congenital (idiots and imbeciles)	31	26	57
Cerebral disease	16	5	21
Childbirth	-----	30	30
Cerebral softening	6	-----	6
Dissipation (alcoholic) and irregular life	110	27	137
Disappointment in love	7	36	43
Domestic troubles	37	53	90
Desertion or death of husband	-----	27	27
Desertion or death of wife	2	-----	2
Epilepsy	66	63	129
Excessive study	4	2	6
Excessive venery	2	5	7
Exposure to cold	10	-----	10
Excessive use of tobacco	1	2	3
Emancipation	5	2	7
Erysipelas	-----	1	1
Fright	4	-----	4
Fever	12	5	17
Gout	1	-----	1
Gun-shot wound of head	1	-----	1
Grief	3	18	21
Gambling	1	-----	1
Hysteria	-----	5	5
Heredity	8	9	17
Ill health	32	40	72
Inordinate venereal desire		6	6
La Grippe	2	-----	2

Fig. 5. Table from 1892 *Annual Report* of “supposed (exciting)” causes of patients’ insanity.

interesting is the fact that the cause of the highest number of “cases” of psychosis in each table (Figs. 5 and 6) is listed as “unknown.” Even though these archival data are interesting, when contextualized, it is not surprising. As mentioned earlier in this text, “causes” for psychosis were often related to physical and social situations, and political control and oppression. And, of course, CSH was not the only state asylum that engaged in these racist practices. In 1848, John Galt, a physician and medical director of the Eastern

TABLE XI. SUPPOSED (EXCITING) CAUSE OF INSANITY OF ALL ADMITTED.			
CAUSES	Males.	Females.	Total.
Loss of property	10	9	19
Loss of eye	2	2	4
Loss of leg	1	-----	1
Masturbation	95	-----	95
Malaria	4	-----	4
Meningitis	2	-----	2
Menopause	-----	20	20
Mammary tumor	-----	1	1
Measles	1	1	2
Old age	21	27	48
Puerperal fever	-----	5	5
Political excitement	29	-----	29
Pregnancy	-----	7	7
Privation	74	52	126
Paralysis	1	2	3
Religious excitement	148	101	249
Sun stroke	7	1	8
Syphilis	13	6	19
Seduction	-----	1	1
Scrofula	5	1	6
Shock from lightning	1	-----	1
Tonsillitis	-----	1	1
Typhoid fever	10	8	18
Uterine troubles	-----	109	109
Unknown	529	508	1,037
Not insane	7	2	9
Total	1,388	1,257	2,645

Lunatic Asylum in Williamsburg, VA, offered that “blacks are immune to mental illness” [33]. In an examination of black mental history, Umech wrote,

Galt hypothesized that enslaved Africans could not develop mental illness because as enslaved people, they did not own property, engage in commerce, or participate in civic affairs such as voting or holding office. This immunity hypothesis assumed according to Galt and others at that time that the risk of ‘lunacy’ would be highest in those populations who were emotionally exposed to the stress of profit making, principally wealthy white men. [33]

Despite the belief that enslaved Africans could not experience mental illness, records from CSH indicate that this did not deter them from being institutionalized. Finally, given what we know about psychiatry and homosexuality, which was classified as a mental illness during this period through the late 20th century, we were surprised to

CAUSES	WITHIN THE YEAR			FROM OPENING OF HOSPITAL 1870		
	Males	Females	Total	Males	Females	Total
Abortion	----	----	----	----	8	8
Apoplexy	----	1	1	7	2	9
Blow on head	10	2	12	151	28	179
Business anxiety	----	1	1	26	1	27
Brain fever	----	----	----	7	3	10
Bright's disease	----	1	3	3	10	13
Burns	----	----	----	2	----	2
Childbirth	----	3	3	----	80	80
Cerebral disease	1	----	1	56	27	83
Congenital (idiots and imbeciles)	----	----	----	41	46	87
Death of child	----	1	1	6	8	14
Death of mother	----	----	----	2	4	6
Degeneration	----	----	----	1	4	5
Desertion of wife	----	----	----	11	----	11
Desertion of husband	----	2	2	----	40	40
Disappointment in love	----	1	1	20	56	76
Domestic troubles	8	2	10	96	131	229
Dissipation (alcoholic and drugs)	18	4	22	340	88	428
Emancipation	----	----	----	5	2	7
Epilepsy	11	8	19	272	148	420
Erysipelas	----	----	----	----	1	1
Excessive study	----	----	----	13	7	20
Excessive venery	----	----	----	10	25	35
Excessive use of tobacco	----	----	----	5	4	9
Exposure to cold	----	----	----	15	4	19
Fever	----	----	----	22	19	41
Fright	----	----	----	11	4	15
Gout	----	----	----	1	----	1
Grief	1	16	17	16	92	108
Gunshot wound in head	----	----	----	2	----	2
Heredity	15	20	35	205	248	453
Hydrocephalus	----	----	----	3	1	4
Hysteria	----	1	1	----	7	7

Fig. 6. Table from 1910 *Annual Report* of "Alleged Cause (as given in commitment papers) of Psychosis of All Admitted."

TABLE IX

Alleged Cause (as given in commitment papers) of Psychosis of All Admitted.

CAUSES	WITHIN THE YEAR			FROM OPENING OF HOSPITAL 1870		
	Males	Females	Total	Males	Females	Total
Ill health	2	9	11	128	130	258
La Grippe	1	----	1	18	9	27
Loss of eye	1	----	1	5	3	8
Loss of leg	----	----	----	3	----	3
Loss of property	1	----	1	28	23	51
Malaria	----	2	2	6	9	15
Marriage	----	----	----	1	4	5
Masturbation	5	2	7	207	7	214
Mammary tumor	----	----	----	----	1	1
Meningitis	----	----	----	12	4	16
Measles	1	----	1	3	1	4
Menopause	----	8	8	----	87	87
Old age	16	10	26	96	99	195
Paralysis	1	1	2	13	9	22
Pregnancy	----	1	1	----	30	30
Privation	1	2	3	125	111	236
Political excitement	----	----	----	32	----	32
Puerperal fever	----	2	2	----	20	20
Religious excitement	23	12	35	262	251	513
Scrofula	----	----	----	5	3	8
Seduction	----	----	----	----	4	4
Shock from lightning	----	1	1	1	1	2
Sunstroke	----	----	----	25	8	33
Syphilis	12	4	16	125	55	180
Tonsillitis	----	----	----	1	1	2
Tuberculosis	----	2	2	3	11	14
Typhoid fever	1	1	2	31	17	48
Uterine disease	----	11	11	----	322	322
Unknown, or not stated	121	68	189	1,581	1,287	2,868
Total	250	201	451	4,069	3,601	7,670

find no references in the tables or narratives to “invert,” “sexual inversion,” or other terms to denote homosexuality or bisexuality in use by mental health professionals during this period. There was also no obvious reference to sexuality as a form of psychosis of those admitted. In Fig. 7, we do find that most diagnoses of psychosis were attributed to general categories of dementia, mania,

and melancholia and the categories are then subdivided to notate different types of dementia, mania, and melancholia. Interestingly, melancholia was defined by Tuke [34] in 1892 as “A disorder characterized by a feeling of misery which is in excess of what is justified by the circumstances in which the individual is placed” [34, p. 787].

TABLE XII.						
Psychosis of Those Admitted						
PSYCHOSIS	WITHIN THE YEAR			FROM OPENING OF HOSPITAL 1870		
	Males	Females	Total	Males	Females	Total
Dementia, epileptic	2	1	3	39	22	61
Dementia, organic	12	2	14	39	24	63
Dementia, parietic	9	-----	9	72	17	89
Dementia, praecox	24	12	36	171	142	313
Dementia, secondary	1	3	4	20	31	51
Dementia, senile	26	28	54	306	261	567
Dementia, (unclassified)*	-----	-----	-----	101	107	208
Epileptic insanity	10	3	13	340	144	484
Exhaustion and infection psychosis	57	18	75	113	63	176
Intoxication psychosis (alcoholic and drug)	9	4	13	136	30	166
Mania, acute**	-----	-----	-----	1,698	1,295	2,993
Mania, chronic**	-----	-----	-----	270	352	622
Mania, hystero**	-----	-----	-----	-----	18	18
Mania, puerperal**	-----	-----	-----	-----	51	51
Mania, recurrent**	-----	-----	-----	122	132	254
Manic-depressive insanity	52	141	193	118	277	395
Melancholia, acute**	-----	-----	-----	319	408	727
Melancholia, agitate	-----	-----	-----	3	16	19
Melancholia, chronic**	-----	-----	-----	22	44	66
Melancholia, involutional	4	5	9	4	5	9
Melancholia, stuporous**	-----	-----	-----	8	14	22
Melancholia, (unclassified)*	-----	-----	-----	102	145	247
Paranoia	5	8	13	104	108	212
Imbecility*	-----	-----	-----	59	48	107
Undetermined	2	-----	2	28	21	49
Not insane while in hospital	4	2	6	92	53	145
Total	217	227	444	4,286	3,828	8,114

Fig. 7. 1911 Annual Report Table XII: Psychosis of Those Admitted.

Further, as Umech [33] notes, Black “insanity” was often connected to the conditions of Black life, both before and after Emancipation. In fact, Umech notes,

Other antebellum medical researchers promoted conditions such as *Drapetomania*, a disease that caused enslaved blacks to flee their plantations, or *Dysaesthesia Aethiopia*, a disease that purportedly caused a state of dullness and

lethargy, which would now be considered depression. Modern historians of slavery have described both conditions as understandable responses to enslavement, but white medical practitioners at the time assumed they were manifestations of mental illness. [33]

Of course, given what we know about Black life before the turn of the 20th century, Tuke’s [34] definition as applied to Black people as an

underlying cause of insanity is disingenuous at best. Even so, beyond the diagnosis of dementia, mania, and melancholia, the primary reason indicated for psychosis in women patients in the 1911 report is manic-depressive insanity (141 women). The footnote to the table (Fig. 7) makes note of Kraepelin's classifications of mental illness after 1910. Kraepelin's later classifications indicated differences between manic-depressive insanity (now called bipolar disorder) and praecox (schizophrenia) [35].

CONCLUSION: SEEING A WAY FORWARD

In taking on this endeavor to learn more about how Black women's mental health was represented in historical archives, we learned a great deal, not from the text on the page of the documents that we were able to access but from the text that was missing from those documents. As Farmer suggests,

First and foremost, ... lack of source material is no longer an adequate justification to marginalize black women—or race and gender analyses—in historical accounts. Recent scholarship indicates that there is ample evidence of their lives and work; historians need only to adjust their methodology in order to better account for the power imbalances embedded in document collections. This reorientation to research is also generative for identifying new sites of historical inquiry. [36, p. 292]

What Ray's productive and inventive reading of absences [19] and Royster's concept of critical imagination [25] deployed together offer us is not definitive proof of the stories of Black women in CSH, but we can read the absences and work toward "interrogating the contexts, conditions, lives, and practices of women who are no longer alive to speak directly on their own behalf" [19, p. 71]. Royster and Kirsch remind us that this is wholly acceptable, this inventing, this imagining, because our goal is not to look for "immutable truth but instead for what is likely or possible, given the facts in hand" [25, p. 71]. Embracing the erasure and recognizing the necessity of critically imaginative and inventive ways of reading absences in archival records allow us, as Black women interested in Black women's "issues," to locate ourselves.

This aligns with new and exciting questions that historians are posing about the implications of Black women's absences from archives. For

instance, Farmer suggests that "newer histories" might focus specifically on the absences as evidence and that scholars might focus on "acknowledging and interrogating issues of empowerment and erasure embedded in the archive itself" [36, p. 289]. Citing Fuentes, Farmer notes the push for

scholars of black women's history to "eke out" the "extinguished and invisible" aspects of their lives by revisiting the known archive and reflecting more deeply on the power imbalances within it. [36, p. 29]

Further, critical imagination paired with rhetorical inventiveness when interrogating absences in the archive allowed us, as Black women scholars, to envision ways of defining ourselves for ourselves. Seeing ourselves in the history records in which we are disappeared and refusing to be defined by the narratives of others is indeed an act of rhetorical resistance.

In her acclaimed novel *Beloved*, Toni Morrison reminds us that "definition belongs to the definers, not the defined" [1, p. 190]. In terms of the disappeared, like Black women in the mental health archives, this disappearance, this lack of definition, presents an opportunity for us, as Black women, to be the definers. However unintentional, we assert that our inventive reading of archival absences allows us to acknowledge that in being unseen, we can deploy critical imagination as a means of being made visible in ways that are authentically our own.

As we came to understand that archival research is quite often about reading absences in archives for exclusion and disappearances, we turn to what this recognition offers technical communication scholars. We assert that technical communication scholars, especially those with an interest in inclusion and justice, must adjust their methodological orientation and their approaches to historical and archival research to

- Consider our proposed critical archival inquiry method for identifying what is missing from what is left of the historical record.
- Embrace interdisciplinary methodology, as described by Walters, who acknowledges that a deeper understanding of the archive can draw on several different genres and document types in a reflexive manner, inviting "us to return to the archive and read anew" [24, p. 7]. For example, Walters notes that

viewing archival materials through the lens of fiction and poetry generates new narratives that challenge our previous understandings of historical events and allows us to see the possibility of resistant identities and tactics, outside the state's constructs. [24, p. 9]

- Question the "textual record as authoritative source of historical fact," as Walters champions, so that we can see and not "miss alternative possibilities" [24, p. 8].
- Acknowledge archives as always rhetorical, not neutral or apolitical in ways that we have also come to understand that the discipline of technical communication is not neutral or apolitical.
- Encounter archives and archival records with productive skepticism that pushes us to engage in the rhetorical invention process and critically imagine the narratives that reside in the absences of a text.

TPC scholars have a unique skill set that is ideal for the reading through absences and erasures in both contemporary and historical documents. Understanding that these historical documents are inherently rhetorical and often technical in nature, we see critical archival inquiry as a productive methodological approach for analyzing historical records in ways similar to Berkenkotter (patient records and narratives), Jones and Williams (literacy tests and voting), and Banks (African American quilts). As a rhetorically informed and culturally sustaining practice, taking up archival work with a focus on rhetorical invention and critical imagination has the potential to present technical communication with a framework well oriented to opportunities and possibilities for pushing the field forward by deploying more inclusive and just ways for engaging with archives.

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